

Drug Management Policy

Stelara

Policy Number: 9.129

Version Number: 5.0

Version Effective Date: 12/1/2024

Impacted Products

- ☐ NH Medicaid
- ☐ NH Medicare Advantage
- ☐ MA MassHealth ACO
- ☐ MA MassHealth MCO
- ☒ MA Clarity Health plans/Employer Choice Direct
- ☐ MA Senior Care Options
- ☒ Pharmacy Benefit
- ☒ Medical Benefit

Note: Disclaimer and audit information is located at the end of this document.

Prior Authorization Policy

Products Affected:

- Stelara® (ustekinumab) subcutaneous injection
- Stelara® (ustekinumab) IV (induction of therapy only)

The Plan may authorize coverage of the above products for members meeting the following criteria:

Covered Use	All medically accepted indications not otherwise excluded
--------------------	---

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

Exclusion Criteria	Use in combination with other biologics
Required Medical Information	Moderate to severely Active Crohn's Disease (CD); AND 1. The member is 18 years of age or older; AND 2. The requested medication is prescribed by or in consultation with a gastroenterologist; AND 3. ONE of the following: a. Member has tried or is currently taking corticosteroids, or corticosteroids are contraindicated in this member; OR b. Member has tried one conventional systemic therapy for Crohn's disease (such as azathioprine, 6-mercaptopurine, methotrexate, or a previous trial of a biologic; a trial of mesalamine does not count); OR c. Member has enterocutaneous (perianal or abdominal) or rectovaginal fistulas; OR d. Member had ileocolonic resection (to reduce the chance of Crohn's disease recurrence); AND 4. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A). 5. The member has or will receive a single induction dose with Stelara intravenous within 2 months of initiating therapy with Stelara subcutaneous. Moderate to severe plaque psoriasis (PsO); AND For the approval of the 45 mg syringe/vial: 1. The member is 6years of age or older; AND 2. The requested medication is prescribed by or in consultation with a dermatologist or rheumatologist; AND 3. ONE of the following:

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

	a. Involvement of at least 3% of total body surface area; OR b. Hands, feet, scalp, face, or genital area affected; AND 4. ONE of the following: a. Inadequate response or adverse reaction to at least a 3-month trial of one biologic DMARD that is FDA-approved for plaque psoriasis; OR b. Inadequate response or adverse reaction to at least a 3-month trial of one conventional therapy (as defined below) in any one of the following combinations (please note: these combinations DO NOT have to be used concurrently): i. One topical agent plus one systemic agent; OR ii. One topical agent plus one phototherapy; OR iii. One systemic agent plus one phototherapy; OR iv. Two systemic agents; OR c. Contraindication to methotrexate, as determined by the prescriber; AND 5. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A). 6. If the 90 mg syringe is requested, approve if the member meets ALL of the above and ONE of the following: a. Member weighs > 100 kg; OR b. Member is currently receiving the 90 mg syringe; OR c. Member has received standard dosing with the 45 mg syringe/vial for at least 3 months with inadequate efficacy. Psoriatic Arthritis (PsA); AND For the approval of the 45 mg syringe/vial: 1. The member is 18 years of age or older; AND 2. The requested medication is prescribed by or in consultation with a dermatologist or rheumatologist; AND 3. ONE of the following:
--	--

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

	a. An inadequate response or adverse reaction to at least a 3-month trial of one non-biologic disease modifying antirheumatic drug (DMARD) or contraindication to all non-biologic DMARDs; OR b. An inadequate response or adverse reaction to at least a 3 consecutive month trial of one biologic DMARD FDA approved for Psoriatic Arthritis; AND 4. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A). 5. If the 90 mg syringe is requested, approve if the member meets ALL the above and ONE of the following: a. Member weighs > 100 kg; OR b. Member is currently receiving the 90 mg syringe; OR c. Member has received standard dosing with the 45 mg syringe/vial for at least 3 months with inadequate efficacy. Moderate to severely active Ulcerative Colitis (UC); AND 1. The member is 18 years of age or older; AND 2. The requested medication is prescribed by or in consultation with a gastroenterologist; AND 3. ONE of the following: a. Member has tried one systemic therapy (such as 6-mercaptopurine, azathioprine, cyclosporine, tacrolimus, a corticosteroid such as prednisone or methylprednisolone, or a previous trial of a biologic); OR b. Member has pouchitis and has tried an antibiotic (such as metronidazole or ciprofloxacin), probiotic, corticosteroid enema, or mesalamine enema; AND 4. Documentation that the requested dose will not exceed FDA-approved dosing (see Appendix A). 5. The member has or will receive a single induction dose with Stelara intravenous within 2 months of initiating therapy with Stelara subcutaneous.
--	---

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

Coverage Duration	Initial: 6 months Continuation: 12 months
Other criteria	Reauthorization: 1. Initial criteria have been met; AND 2. One of the following: a. The member has had a positive response to therapy; OR b. Documentation of clinical justification to continue therapy with the requested medication

Appendix A – FDA Approved Dosing

Diagnosis	FDA Approved Dosing
Crohn Disease	<u>Adult:</u> Induction: IV: ≤55 kg: 260 mg as single dose, >55 kg to 85 kg: 390 mg as single dose, >85 kg: 520 mg as single dose Maintenance: SUBQ: 90 mg every 8 weeks, begin maintenance dosing 8 weeks after the IV induction dose.
Plaque psoriasis	<u>Adult:</u> ≤100 kg: 45 mg at 0 and 4 weeks, and then every 12 weeks thereafter. >100 kg: 90 mg at 0 and 4 weeks, and then every 12 weeks thereafter. <u>Pediatric:</u> <u>Children ≥6 years and Adolescents <18 years:</u> <60 kg: SUBQ: 0.75 mg/kg at 0 and 4 weeks, followed by maintenance dose of 0.75 mg/kg every 12 weeks. ≥60 to ≤100 kg: SUBQ: 45 mg at 0 and 4 weeks, followed by maintenance dose of 45 mg every 12 weeks. >100 kg: SUBQ: 90 mg at 0 and 4 weeks, followed by maintenance dose of 90 mg every 12 weeks.

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

	<u>Adolescents ≥ 18 years:</u> ≤ 100 kg: SUBQ: 45 mg at 0 and 4 weeks, followed by maintenance dose of 45 mg every 12 weeks. > 100 kg: SUBQ: 90 mg at 0 and 4 weeks, followed by maintenance dose of 90 mg every 12 weeks.
Psoriatic arthritis	<u>Adult:</u> Initial and maintenance: 45 mg at 0 and 4 weeks, and then every 12 weeks thereafter. Coexistent psoriatic arthritis and moderate to severe plaque psoriasis in patients > 100 kg: Initial and maintenance: 90 mg at 0 and 4 weeks, and then every 12 weeks thereafter. <u>Pediatric:</u> Children ≥ 6 years and Adolescents < 18 years: < 60 kg: SUBQ: Initial: 0.75 mg/kg at 0 and 4 weeks, followed by maintenance dose of 0.75 mg/kg every 12 weeks. ≥ 60 kg: SUBQ: 45 mg at 0 and 4 weeks, followed by maintenance dose of 45 mg every 12 weeks. > 100 kg and moderate to severe plaque psoriasis comorbidity: SUBQ: 90 mg at 0 and 4 weeks, followed by maintenance dose of 90 mg every 12 weeks.
Ulcerative colitis	<u>Adult:</u> Induction: IV: ≤ 55 kg: 260 mg as single dose, > 55 kg to 85 kg: 390 mg as single dose, > 85 kg: 520 mg as single dose. Maintenance: SUBQ: 90 mg every 8 weeks; begin maintenance dosing 8 weeks after the IV induction dose.

Appendix B - Non-Biologic DMARD Treatment Options

Diagnosis	Non-Biologic DMARD Treatment Options
Plaque Psoriasis	Methotrexate Azathioprine Cyclosporine

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

Psoriatic Arthritis	Methotrexate Leflunomide Sulfasalazine Azathioprine
---------------------	--

Note: other trials may be considered on a case-by-case basis

Applicable Coding:

Code	Medication
J3357	ustekinumab, for subcutaneous injection, 1 mg.
J3358	ustekinumab, for intravenous injection, 1 mg

Clinical Background Information and References

1. American Academy of Dermatology Association. Psoriasis Clinical Guideline.
<https://www.aad.org/member/clinical-quality/guidelines/psoriasis>. Accessed June 2024
2. Elmets, C. Korman, N. Farley Prater, E. et. al. Joint AAD–NPF Guidelines of care for the management and treatment of psoriasis with topical therapy and alternative medicine modalities for psoriasis severity measures. Journal of the American Association of Dermatology. 84(2): p432–470. Feb. 2021. Accessed June 2024
3. Elmets, C. Lim, H, Wu, J. et. al. Joint American Academy of Dermatology–National Psoriasis Foundation guidelines of care for the management and treatment of psoriasis with phototherapy. Journal of the American Association of Dermatology. 81(3): p775–804. Sept. 2019. Accessed June 2024.
4. Feuerstein, JD, Isaacs KL, Schneider Y, et al. AGA Clinical Practice Guidelines on the Management of Moderate to Severe Ulcerative Colitis. Gastroenterology. 2020; 158:1450–1461. Accessed June 2024.
5. Feuerstein, Joseph D., et al. "AGA Clinical Practice Guidelines on the Medical Management of Moderate to Severe Luminal and Perianal Fistulizing Crohn’s Disease." Gastroenterology, vol. 160, no. 7, June 2021, pp. 2496–508, <https://doi.org/https://doi.org/10.1053/j.gastro.2021.04.022>. Accessed July 2024.
6. Galdman, Orbai A. Treatment of psoriatic arthritis. UpToDate. May 2024. Accessed June 2024.
7. Hashash JA, Reguerio M. Medical management of moderate to severe Crohn disease in adults. UpToDate® available at <https://www.uptodate.com>, accessed June 2024.

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

8. Menter, A. Strober, B. Kaplan, D. et. al. Joint AAD-NPF guidelines of care for the management and treatment of psoriasis with biologics. Association of Dermatology. 80(4): p1029-1072. April 2019. Accessed June 2024.
9. Reguerio M, Hashash JA. Overview of the medical management of mild (low risk) Crohn disease in adults. UpToDate® available at <https://www.uptodate.com>, accessed June 2024.
10. Singh, J. Guyatt, G. Gladman, D. 2018 American College of Rheumatology/National Psoriasis Foundation Guideline for the Treatment of Psoriatic Arthritis. Arthritis & Rheumatology. 71(1): p. 5-32. January 2019. Accessed June 2024.
11. Stelara (ustekinumab) [package insert]. Horsham, PA: Janssen Biotech, Inc.; Accessed July 2024.

Original Approval Date	Original Effective Date	Policy Owner	Approved by
12/1/2020	1/1/2021	Pharmacy Services	Pharmacy & Therapeutics (P&T) Committee

Policy Revisions History			
Review Date	Summary of Revisions	Revision Effective Date	Approved by
12/1/2020	9.193 Stelara Policy retired, new policy created. Updated age expansion for PS; updated trial and failure requirements for some indications	1/1/2021	P&T Committee
4/23/2021	Removed trial requirement of Humira for diagnosis of Ulcerative Colitis (UC) to align with ESI ICCV	4/23/2021	P&T Committee
8/12/2021	P&T Annual Review: Addition of 3 month requirement for most trials; Addition of criteria to all indications to allow a previous trial of a biologic DMARD to bypass non-biologic DMARD criteria; Criteria updates for CD and UC to align with ESI ICCV policy guidelines and trial requirements; Addition of meeting initial criteria to reauthorization guidelines; Addition of diagnostic criteria for Ps.	12/1/2021	P&T Committee

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

Stelara

Policy Revisions History			
8/11/2022	P&T Annual Review: Addition of documentation language to trial and failure criteria. Change initial approval duration from 12 to 6 months.	12/1/2022	P&T Committee
8/10/2023	P&T annual review: Addition of criteria for 90mg dosing for diagnoses of PS and PsA. Update reauthorization criteria to align with other drug policies in its class.	12/1/2023	P&T Committee
8/8/2024	P&T annual review: No recommended changes	12/1/2024	P&T Committee

Next Review Date

8/2025

Other Applicable Policies

Reference to Applicable Laws and Regulations, If Any

Disclaimer Information

Medical Policies are the Plan's guidelines for determining the medical necessity of certain services or supplies for purposes of determining coverage. These Policies may also describe when a service or supply is considered experimental or investigational, or cosmetic. In making coverage decisions, the Plan uses these guidelines and other Plan Policies, as well as the Member's benefit document, and when appropriate, coordinates with the Member's health care Providers to consider the individual Member's health care needs.

Plan Policies are developed in accordance with applicable state and federal laws and regulations, and accrediting organization standards (including NCQA). Medical Policies are also developed, as appropriate, with consideration of the medical necessity definitions in various Plan products, review of current literature, consultation with practicing Providers in the Plan's service area who are medical experts in the particular field, and adherence to FDA and other government agency policies. Applicable state or federal mandates, as well as the Member's benefit document, take precedence over these guidelines. Policies are reviewed and updated on an annual basis, or more frequently as needed. Treating providers are solely responsible for the medical advice and treatment of Members.

* Plan refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.

The use of this Policy is neither a guarantee of payment nor a final prediction of how a specific claim(s) will be adjudicated. Reimbursement is based on many factors, including member eligibility and benefits on the date of service; medical necessity; utilization management guidelines (when applicable); coordination of benefits; adherence with applicable Plan policies and procedures; clinical coding criteria; claim editing logic; and the applicable Plan – Provider agreement.

^{*} *Plan* refers to Boston Medical Center Health Plan, Inc. which operates under the trade name WellSense Health Plan.